Case presentation
Paediatrics

Presented by
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Core member of whitearmy

Demographic information

- Name-Mr ABC
- Age-10months
- Sex –male
- Adress-malalli,kundgol taluk
- Religion- hindu

- Informant – Ms xyz(mother)
- Age-24yrs
- Education-3rd standard

- Date of admission-23/02/2020
- Date of examination-27/02/2020

Chief complaints

- Fever for 5days
- Loose stools for 4days

History of present illness

- Child was apparently alright 5 days back the the child developed fever, continuous type, Moderate degree, not associated with rigors or any rashes, No diurnal variation and relived on medication

- Mother also complains of passage of loose stools by The baby since 4 days which was sudden In onset, watery in consistency, Non foul smelling, initially 5-7 episode per day later increased to 8-10 episode per day , yellowish in colour, non blood tinged,scanty in amount no passage of worms in stool

- Mother gives history excessive thirst in baby and sunken eyeball which relieved after coming to hospital

- No history of vomiting
- No history of any precipitating food intake
- No history of distention of abdomen
- No history of crying during micturition
- No history of edema ,baggy skin, Decreased activity
- No history of any drug intake
- No history of decreased urine output
- No history of weakness, convulsions

Past history

- No similar complaints in the past
- No history of previous surgery

Birth history

Antenatal history

- Booked case uneventful
- Anomaly scan was done
- Iron folic acid tablet taken
- No history of fever with rashes
- No history of antepartum haemorrhage
- No exposure to radiation

Natal history

- Full term vaginal delivery At 9 months 16 days
- Birth weight-2.4kg Cried immediately after birth
- Urine and meconium passed after 2 hours of birth
- No history of admission to NICU
- Breast feeding started within an hour after birth

Postnatal history

- No history of neonatal Jaundice
- Exclusivelyly breastfed for 6months and continued for 1year
- Supplementary food was given after 6 months
- No history of any infection

Developmental history

Gross

- Neck control at -3 month
- Roll over- 6month
- Sitting with support-7 month
- Standing with support -9 month

Fine motor

- Bidextrous grasp -4 month
- Unidextrose grasp -6month
- Mature pincer grasp-9 month

Social

- Recognised mother-3 month
- Waves bye bye -9month

Language

- Speak monosylabals-6 month

Developmental milestones achieved regularly

Immunization history

- Immunized till date
- Last vaccine -MR,JE, Vitamin A at 9month
- Rota virus vaccine not taken

Nutritional history

- Morning- 1 banana -110k cal- 1.g Protein
- 4 Biscuits - 80 kcal – 1g protein
- Afternoon – 1 glass Milk-160 k cal -8 g

. Ragi malt. -110k cal -2g

- Evening -4 biscuits-80 k cal 1g protien
- Night – Smashed rice and vegetables-110k cal -2g protein

Total- 650k cal ,15g protein

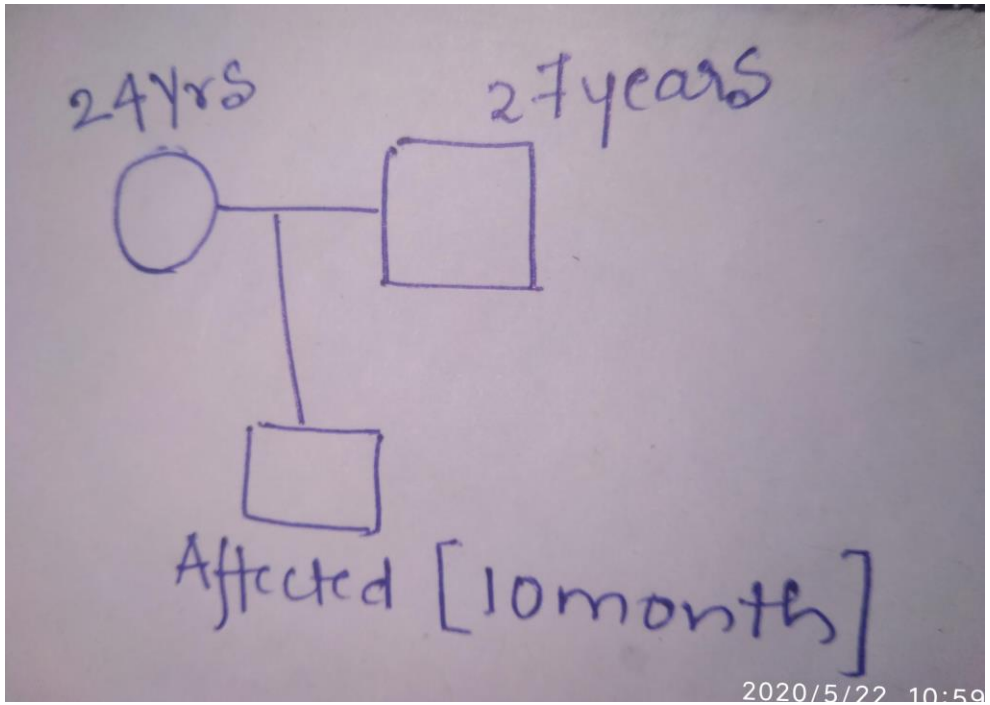
Required – 800 cal ,16g

Deficient- 250 k cal ,1g

Percentage – 30%in calories, 6.25% in protien

Family history

- Non consanguineous marriage
- Married life -2 years
- No similar Complaints in other family members



Socioeconomic status

- Father is a farmer And studied till SSLC
- Belongs to class 4 socioeconomic status according to kuppuswamy classification
- Lives in a Kucha house with overcrowding and with good ventilation
- No open defication , drink filtered water water supplies from municipal corporation they store in vessel use dipping type

- **Summary**

Here is a 10 month old male child who is born to a non consanguineous married couple belonging to class 4 socioeconomic status ,who consumes a diet Which is 30% deficient in calories and 6.25% deficient in protien ,who is immunized upto date with normal developmenal milestones and with no similar complaints in past. Now comes with chief complaint of , fever since 5 days ,loose stools since 4 days

EXAMINATION

General survey

Here is a 10month old male child which is alert,active and playful

Vital signs

Temperature- 98.5° farenhiet measured in axilla

Pulse -114 beats per minute good volume, no radio radial or radio femoral delay – normal

Respiratory rate-38cycles per minute- normal

Blood pressure-70/50mm Hg

Head to toe examination

Sign of dehydration

Tears present

Tongue moist

Not thirsty

Skin pinch –retracts immediately

- Hair –normal
- Anterior fontanel- appear to be sunken
- Posterior fontanel – closed
- Eye- pallor is present

. Icterus is not present

. No features of vitamin A deficiency

- Nose- normal
- Teeth –normal
- Oral cavity- Good Hygiene no features of anaemia

- Nails – normal
- Skin – On pinching the skin, it goes back immediately
- - Mangolian spots present on back
- Chest ,Spine and back normal

- Pelvis- There is loss of pad of fat in groin
- No cyanosis, clubbing, lymphadenopathy,edema

Anthropometry

	Observed	Expected	Inference
Weight	6.7 kg	10 .7kg	<-3 SD
Length	71 cm	76cm	<-1 SD
Head circumference	44 cm	45cm	Normal
Mid arm circumference	12cm	>13.5cm	<-2 SD

- **Systemic examination**

Per abdomen examination

INSPECTION

- Abdomen is distended Uniformly
- Umballicus central and everted
- Corresponding quadrants move equally with respiration
- No dialated veins Present ,no scars, no pulsation
- Hernial orifices normal

- External genitalia normal

PALPATION

- No local rise of temperate,no tenderness
- On Deep palpation
- Abdomen is soft ,no organomegaly appreciated

PERCUSSION

- Tympanic note heard

AUSCULTATION

- Normal bowel sounds heard

- **Other systems**

- **CVS-**

  -No raised JVP, S1 S2 heard no murmur

- **RS –**

. Normal vesicular breath sounds heard Bilateral equal air entry no added sounds

- **CNS-** No altered sensorium , sensory and motor system examination is normal

- **DIAGNOSIS**

Case of Acute diarrhoeal disease of infective pathology With No dehydration and With no impending complications

THANK YOU